CASCADE ORTHOPEDIC & SPINE - NEW PATIENT CONSULTATION
Patient: Morgan Ellis DOB: 04/11/1985
Date of Service: 02/24/2026 Physician: Sofia Ramirez, MD, Orthopedic Spine Surgery
Referring Provider: Alan Brooks, MD

CHIEF COMPLAINT: Neck pain with arm symptoms since a motor vehicle collision on 01/09/2026.

HISTORY OF PRESENT ILLNESS:
Ms. Ellis is a 40-year-old warehouse shift supervisor who was the restrained driver of a stopped vehicle that was rear-ended by a box truck on 01/09/2026. She developed neck pain immediately and was evaluated at Mercy General ED, where CT of the head and cervical spine showed no fracture. She has completed about 5 weeks of physical therapy with modest improvement in range of motion but persistent pain. Over the past 3 weeks she has developed radiating pain from the neck into the right arm, with numbness in the right thumb and index finger. Pain is 6/10 in the neck and 5/10 in the arm, worse with looking up and with prolonged computer use. She is dropping objects from her right hand. She has been unable to return to work. She denies any prior neck problems or treatment before this collision.

PAST MEDICAL HISTORY: Seasonal allergies.
MEDICATIONS: Naproxen 500 mg twice daily, cyclobenzaprine 10 mg at bedtime, loratadine.

PHYSICAL EXAMINATION:
Cervical spine: Paraspinal tenderness C4-C7. Rotation 60 degrees right, 50 degrees left. Extension reproduces arm pain. Spurling test positive on the right.
Motor: Right wrist extension 4/5, right biceps 4+/5; otherwise 5/5.
Sensation: Decreased to light touch in the right C6 dermatome (thumb and index finger).
Reflexes: Right brachioradialis 1+, left 2+. Hoffmann sign negative bilaterally.
Gait: Normal.

IMAGING REVIEWED:
MRI cervical spine 02/10/2026 (Summit Imaging): C5-C6 disc protrusion with annular fissure and foraminal narrowing contacting the exiting C6 nerve root. CT cervical spine 01/09/2026: no fracture.

ASSESSMENT:
1. Right C6 cervical radiculopathy secondary to C5-C6 disc protrusion with annular fissure.
2. Cervical strain.
3. Lumbar strain, improving.

CAUSATION:
Based on the history, the timing of symptom onset, the absence of prior neck complaints, and the imaging findings including an annular fissure, it is my opinion within a reasonable degree of medical probability that the C5-C6 disc injury and resulting radiculopathy were caused by the motor vehicle collision of 01/09/2026.

PLAN:
1. Referral to Riverbend Pain Specialists for cervical epidural steroid injection.
2. Continue physical therapy with emphasis on cervical stabilization and nerve glides.
3. Gabapentin 300 mg at bedtime, titrate to three times daily as tolerated.
4. Remain off work. No lifting over 10 pounds, no overhead work.
5. If symptoms persist after injections and therapy, discuss surgical options including anterior cervical discectomy and fusion at C5-C6.
6. Follow up in 10 to 12 weeks.

Electronically signed: Sofia Ramirez, MD 02/24/2026